Storage Condition :

Store in a dry place (below 30^{o}C). Keep container tightly closed. Protect from light.

Pack Size :
Blister pack of 1x10's, 3x10's and 10x10's.

Registration Number :
MAL19963144X

Further information can be obtained from your doctor or pharmacist.

ROYCE®

Product Holder / Manufactured by:
ROYCE PHARMA MFG. SDN.BHD. (650435-X)
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ROYCE®

Polysil Tablet

(Aluminium Hydroxide, Magnesium Hydroxide and Simethicone – Antacid)

Presentation:
A pink colour, round, flat, bevel-edged tablet with 16mm in diameter and peppermint flavour.

Each tablet contains:
Dried Aluminium Hydroxide Gel 250mg, Magnesium Hydroxide 250mg and Polymethylsiloxane (Simethicone) 50mg.

Indication:
Gastric hyperacidity, peptic ulcer, heartburn and flatulence.

Pharmacological Information:
Aluminium hydroxide and Magnesium Hydroxide reacts with hydrochloric acid in the stomach to form aluminium chloride and magnesium chloride respectively. Thus, resulting a relief from the acid pain.

The insolubility of aluminium hydroxide is a desirable property in that a fraction of sedimented unreacted excess tends to remain in the stomach to yield a sustained effect.

Unreacted $Mg (CH)_2$ remains in the stomach to react with acid subsequently secreted. Therefore, it also has a some-what prolonged duration of action.

Magnesium salts act as saline laxatives in the intestine while aluminium hydroxide has a constipating effect. Thus, these 2 compound complement each other in the same preparation. Activated polymethysolixane changes the surface tension of gas bubbles, thereby causing then to coalesce. It is used in the treament of flatulence, for the elimination of gas or air from the gastro-intestinal tract, and for the relief of abdominal distention and dyspepsia.

The insoluble aluminium compounds are slowly and perhaps incompletely converted to a aluminium chloride in the stomach. Mucus secretion is supposedly stimulated by the irritant of Al^{3+}. Some absorption of soluble aluminium salts occurs from the gastro-intestinal tract with some excretion in the urine. Some unabsorbed aluminium hydroxide combines with phosphates present in the gut to form insoluble aluminium phosphates and some forms carbonates and salts of fatty acids. All this salts are excreted in the faeces.

The neutralizing action of magnesium hydroxide is very prompt and complete. 5 to 10% of he magnesium can be absorbed. The absorbed magnesium ion is rapidly excreted by the kidney. In normal persons, absorption is attended by little or no danger of systemic alkalosis, but the urine may become alkaline.

Dosage and administration :
Oral administration.
Dosage: 1 or 2 tablets to be chewed between meals and at bedtime, or as required

Contraindication:
With the exception of hypophosphataemia, there are no known contraindications to Polysil.

Warning and Precautions :
Proteins, peptides, amino acids, and certain dietary organic acids greatly impair the neutralizing capacity of a aluminium hydroxide.

Possibility of mobilizing bone salts on long terms of therapy. Aluminium hydroxide could cause white discoloration of the feaces. As with other drugs, the administration of Polysil during the first trimester of pregnancy is only advised if there are compelling reasons for their use.

Usage during Pregnancy and Lactation:
For Aluminium and magnesium – containing antacids: They are considered safe for use during the last two trimesters of pregnancy as long as chronic high doses are avoided.

For Simethicone:
Problems in humans have not been documented; although some aluminium and magnesium may be excreted in breast milk, the concentration is not great enough to produce an effect in the neonate.

Drug Interactions:
Not available.

Side Effects:
A phosphate-depletion syndrome is occasionally observed after chronic aluminum hydroxide ingestion. Patients on a therapeutic regiment for peptic ulcer usually have a high phosphate intake, so that binding of phosphate is of no consequences. However, if the phosphate intake is limited, phosphate deprivation can occur. Decreased phosphate absorption is accompanied by increased calcium absorption, which along with resorption of bone salts, causes hypecalciuria (see lotz et al., 1968; Ansari, 1970). Hypomagnesemia and hypomagneriuria also occur.

Aluminium ion forms coordinate complexes with a wide variety of substances. Reactions with proteins account for its astringent properties.Some individuals are intolerant of the astringent action of the drug and experience nausea and vomiting. Concretions of fatty acid salts of aluminium may occur in the stools. Several cases of intestinal obstruction by a large mass composed of clotted blood and Aluminium hydroxide have been reported.

Symptoms and Treatment of Overdose:
The components of Polysil Tablets are not expected to cause either local systemic signs of poisoning, even in cases of extreme oral overdosage.